

Cognitive decline & dementia support

NOVA-E2 2 cc intranasal via catheter · NOVA-M1 1-2 cc deltoid IM

An adjunctive framework for mild cognitive impairment or diagnosed dementia. Intranasal delivery through the olfactory and trigeminal pathways bypasses the blood-brain barrier and may support reduction of neuroinflammation and neuronal repair signaling. It does not replace disease-specific medical management.

INTRANASAL

2 cc

1 cc per nostril, NOVA-E2

DELTOID IM

1-2 cc

NOVA-M1

CHAIR TIME

~40 min

two nostrils, 10 min apart

REPEAT

4-6 wk

by symptom severity

Patient selection

- Documented mild cognitive impairment or dementia diagnosis with baseline cognitive testing on file
- Caregiver present to consent where needed and report changes
- Medically stable, able to sit with the head tilted back for 10 minutes at a time
- Clear nasal passage: no active sinus infection, severe congestion, or recent nasal surgery

Exclusion

- Active sinus or respiratory infection, or recent nasal or sinus surgery
- Severe deviated septum without ENT clearance
- Pregnancy unless approved by the Medical Director
- Agitation or delirium that prevents safe positioning — reschedule rather than restrain

Administration & aftercare

SEQUENCE — FULL TECHNIQUE ON PAGE 08

- 1 Seat the patient, head tilted back 30-45°. Draw 2 cc into two syringes and attach the catheter per manufacturer instructions.
- 2 Left nostril: instill 1 cc over 10-20 seconds, hold the position 8-10 minutes, then wait 10 minutes.
- 3 Right nostril: repeat, holding another 8-10 minutes.
- 4 Administer 1-2 cc NOVA-M1 into the deltoid, then observe 5-10 minutes.

OUTCOME MEASURES

MoCA OR MMSE

CAREGIVER-REPORTED ADLs

SLEEP QUALITY

AGITATION FREQUENCY

Record the same instrument at baseline and at the 6-week visit. Caregiver observations are part of the record, not an aside.

POST-PROCEDURE INSTRUCTIONS

- No nose blowing for at least 60 minutes
- Avoid bending forward for 20-30 minutes
- Hydrate normally; no alcohol the same day
- Mild throat drainage is expected and harmless

FOLLOW-UP

First follow-up at **2 weeks**, second at **6 weeks**. Repeat every 4-6 weeks by symptom severity, re-evaluating after 2-3 sessions.

RISKS & SAFETY

Sinus discomfort or pressure, transient headache, throat drainage, and epistaxis are the expected complaints. Monitor for sinus infection after the visit. Continue all standard dementia care, including prescribed medication and cognitive or behavioral support.

NO CURE CLAIM

Nothing in this protocol is offered as a treatment for Alzheimer's disease or any other dementia, and no claim of slowing, halting, or reversing cognitive decline is made. Response varies and may be absent. Discuss expectations with both the patient and the caregiver before consent.

PATIENT NAME	DATE OF BIRTH	PATIENT ID / MRN	DATE OF VISIT
EVALUATING PHYSICIAN	INITIAL · FOLLOW-UP VISIT TYPE — CIRCLE	REFERRING PROVIDER, IF ANY	TIME IN / OUT

S SUBJECTIVE

Cognitive decline / memory concerns —
 CHIEF COMPLAINT

ONSET / DURATION

SEVERITY — VAS 0-10

QUALITY / RADIATION

AGGRAVATING FACTORS

RELIEVING FACTORS

Workup on file: labs · imaging · neurology or geriatric eval — confirm
 PRIOR TREATMENT — CIRCLE ALL TRIED · ADD DATES

ADLs · orientation · word-finding · behavior changes per care pa...
 FUNCTIONAL IMPACT

fever · chills · weight loss · neuro changes — note positives
 PERTINENT ROS

MEDICATIONS — INCL. ANTICOAGULANTS, IMMUNOSUPPRESSANTS

ALLERGIES · RELEVANT PMH — CANCER, INFECTION, PREGNANCY

O OBJECTIVE

BP

HR

TEMP

WEIGHT

GENERAL APPEARANCE

Mental status — orientation, attention, recall

MoCA or MMSE — score

Cranial nerves and motor screen

Gait and fall risk

Nasal passages — patency

IMAGING / STUDIES REVIEWED — MODALITY, DATE, FINDINGS

BASELINE — MoCA/MMSE

BASELINE — ADL scale

BASELINE — Caregiver log

A ASSESSMENT

MCI or dementia — diagnostic workup documented on file
 WORKING DIAGNOSIS — CONFIRM OR AMEND

ADDITIONAL DIAGNOSES / CODES

☐ Appropriate candidate — indications met, contraindications absent

☐ Alternatives discussed, including conservative care and doing nothing

☐ Non-FDA-approved status explained; no outcome promised

☐ Patient questions answered; understanding confirmed

P PLAN

Proposed treatment. Intranasal NOVA-E2 2 cc in divided aliquots plus NOVA-M1 2 cc deltoid IM, 23 G, with care partner present. Observation 15-30 min. Caregiver log issued and explained.

☐ Informed consent (CN-PC) reviewed and signed

☐ Procedure performed today — see CN-OP-DEM · or scheduled (date right)

☐ Fees, non-coverage by insurance, and refund policy reviewed

☐ Follow-up: review at 4-6 wk with caregiver log

PHYSICIAN SIGNATURE	PRINTED NAME & LICENSE NUMBER	PROCEDURE DATE, IF SCHEDULED
---------------------	-------------------------------	------------------------------

PATIENT NAME

DATE OF BIRTH

PATIENT ID / MRN

DATE OF PROCEDURE

PERFORMING PHYSICIAN

MCI or dementia — diagnostic workup documented o...

PRE-PROCEDURE DIAGNOSIS — CONFIRM OR AMEND

START TIME

ASSISTING STAFF

PRE-PROCEDURE CHECKLIST

Check each box before the first draw

☐ Informed consent (CN-PC) signed and on file; SOAP note (CN-SN-DEM) complete

☐ Screening reviewed — no contraindications present

☐ Time-out performed — correct patient, site, product, route

☐ Five checks read aloud with second staff: ID · product & lot · expiry · volume & route · consent

☐ Baseline outcome measures recorded: MoCA or MMSE · caregiver log · ADLs

☐ Site marked and prepped — chlorhexidine / povidone-iodine (circle); chain-of-custody log initiated

BP · HR · TEMP

agent · concentration · volume — write NONE i...

BP · HR · TEMP

PRE-PROCEDURE VITALS

LOCAL ANESTHETIC

POST-OBSERVATION VITALS

PRODUCTS ADMINISTERED

Care partner present for instructions and observation

PRODUCT & TARGET	PLANNED	LOT	EXPIRY	GIVEN
NOVA-E2 (exosome) Intranasal via atomizer or soft catheter, both nares	2 cc			
NOVA-M1 Deltoid IM	2 cc			

TECHNIQUE

Position: seated, reclined; care partner present

- Small aliquots per naris, alternating sides; remain reclined 10–15 minutes after the final aliquot.
- Deltoid IM component: aspirate before injecting, inject slowly.
- Baseline cognitive score on file; caregiver log issued and explained before discharge.

BOTH NARES + DELTOID L · R — circle deltoid side
SITE / SIDE

ATOMIZER · SOFT CATHETER — circle
GUIDANCE

n/a intranasal · 23 G IM

target 15–30 min

NEEDLE USED

OBSERVATION — MINUTES

NARRATIVE — FINDINGS, ASPIRATE, DEVIATIONS FROM PROTOCOL, PATIENT RESPONSE

POST-PROCEDURE & DISPOSITION

Tolerated: WELL · WITH EVENTS — circle

Discharged: STABLE · AMBULATORY · WITH ESCORT — circle

☐ Patient note CN-PN-DEM reviewed and given at discharge

COMPLICATIONS / ADVERSE EVENTS — WRITE "NONE" IF NONE

Any adverse event also requires the adverse-event response steps in this book.

Follow-up plan. Repeat consideration at 4–6 weeks; caregiver log and cognitive screen reviewed at every visit.

FOLLOW-UP SCHEDULED FOR

PHYSICIAN SIGNATURE

PRINTED NAME & LICENSE NUMBER

DATE & END TIME

Today you received an exosome product given as a fine mist through the nose, plus one intramuscular cell injection in the arm. This note is for you and your care partner. The products used are made from donated birth tissue and are not FDA-approved treatments. This page tells you what is normal in the days ahead, how to care for yourself, and when to call us.

DATE OF TREATMENT

SITE(S) TREATED

NEXT VISIT

WHAT TO EXPECT

Changes are gradual — do not judge the result in the first days

TODAY

A drip at the back of the throat, an odd taste, or mild congestion is normal, plus some arm soreness. The rest of the day can follow the usual routine.

FIRST WEEKS

Any change is gradual. Keep routines, sleep, and hydration steady, and note observations in the caregiver log rather than judging day to day.

BY FOLLOW-UP

Bring the log. Progress is measured against the baseline testing, not against memory of a good or bad day.

DO

- Keep the daily routine and sleep schedule steady
- Care partner: a one-line note in the log each day
- Continue all prescribed medications unchanged
- Drink water normally through the day

AVOID

- Blowing the nose forcefully for 4–6 hours
- Nasal sprays for 24 hours unless prescribed
- Alcohol for 72 hours
- Judging results in the first days

CALL US RIGHT AWAY IF

For an emergency, call 911 first

- Confusion suddenly worse than the usual baseline
- A fall, or any head injury
- Fever above 101 °F, or a nosebleed that does not stop
- New weakness or trouble speaking — call 911

CLINIC PHONE

AFTER-HOURS PHONE

NOTES FROM YOUR VISIT